Fear (from Seven Signs of Life)

By Aoife Abbey

(Pgs 17-31) If someone had asked me in medical school what I might be most afraid of, dead people wouldn't even have made the list. The first body I ever saw was when my grandmother died. She was laid out in her best clothes on top of her own duvet on her own bed. I remember being 11 years old and sitting on the bed beside my cousin, chatting, whilst absent-mindedly rearranging the rosary beads around her cold, dead fingers.

INTRODUCTION

Grief. Anger. Hope. Joy . . . Fear. All emotions we expect to encounter over our lifetime. But what if this was every day? And what if your ability to manage them was the difference between life and death? For a doctor in Intensive Care this is part of the job. And the fear in the eyes of a terminally ill patient who pleads with you to not let them die can be overwhelming.

In the words of Nelson Mandela: I learned that courage was not the absence of fear, but the triumph over it ...

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My first job as a doctor was on an elderly care ward and, for obvious reasons, it didn't take long until the issue of a dead patient presented itself. I don't think I had even considered it might be my job to 'verify' death. I entered the room alone and shut the door behind me. I approached the bedside and when I placed my stethoscope on her chest, I could hear those hollow noises that rumble inside the thorax of somebody who is dead - the sort of noises that make a newly qualified doctor think: What if they're not dead at all? What if I say they are dead, and then the family walk in the door and she moves?

Verification of death involves looking in the pupils, checking for a response to pain and stating that there is no respiratory effort, no palpable pulse and no heart sounds. I stood for the requisite 2 minutes with my stethoscope on her chest and my hand on her neck, where you would find a carotid pulse in a living person. I became convinced that she was going to open her eyes. I pictured them snapping open, her hand lurching towards my wrist, clamping shut with a strength that I knew she did not have in recent life. I let my imagination construct

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the whole terrifying ordeal; I couldn't stop it, and when the second-hand signaled the passing of 2 minutes, I raced out of the room, heart pounding.

I don't know if I've ever experienced undiluted fear quite like that again, though I do think that at least some of what I do requires a level of courage. And how can I call myself brave, if I cannot admit to still feeling fear?

It reminds me of something . . . In my childhood home, we had a staircase, and the first seven steps had empty spaces between the treads. As a child, I would convince myself that a hand was about to spring from one of those holes and grab my ankles. It wasn't real, I knew it wasn't, but I let myself believe it and I ran so fast up those stairs. Perhaps it is true that we create most of the fear we experience.

George Eliot said that 'we learn words by rote, but not their meaning; that must be paid for with our life-blood, and printed in the subtle fibers of our nerves'. From the first day that I started working as a doctor, I knew what a 'Do not Resuscitate' (DNR) order was. But more than 3 years into my career, I stood with a patient and a DNR

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order, and fear made me question everything I knew.

It was 4 a.m. and I had found myself on the way to the oncology ward to see a woman in her fifties. She was about the same age as my own mother and was very short of breath. I still recall thinking how calm and composed she was, for somebody who was struggling so obviously. I enquired if the woman had any pain. She shook her head. 'You don't have ***any*** pain in your chest? Can you slow your breathing down?' I placed my stethoscope on the patient's back and listened to her breath sounds; air moved in and out of her lungs, and they sounded clear. 'I'll be back,' I said, as I left her behind me to look at the ECG, which had been placed on the nurses' station outside. I picked up her folder and there, poking out of the front cover, was the DNR order. It had been discussed and signed just two days before, citing metastatic cancer. I remembered only in that instant that I was on the oncology ward.

The nurse called from the patient's bedside, 'You better come back in here, Doctor.' My patient lay flat on top of her bed: breathing, heart beating, but unresponsive.

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Seconds ticked past and I was helpless, standing there thinking that I owed her something, but she was dying so abruptly. The nurse said she would go and call her family, although the patient would probably be dead before they arrived. I couldn't bring myself just to leave this woman alone.

In that moment, time surrounded me like an army advancing: step, step, the sound of heavy boots on ground, rifles brandished with their bayonets pointed in. Was there even a minuscule chance that there was something else that could be done, beyond standing there and watching her die? For a doctor who was scared, yes, it was the right thing for me. Within seconds, I felt my patient's pulse slip away and then she was dead. Was it the right thing for my patient? I don't believe it was, entirely. The best thing would have been for me to hold her hand and be present with her for her death. That wasn't the doctor she got that night; I did not have that presence of mind, and I don't mind admitting it.

Doctors have coping strategies to explain away fear. Mostly this happens unconsciously - the goal is to

4NG Fanatics

micromanage it. You cannot be a rabbit in the headlights - it simply isn't helpful. As an intensive-care doctor, my patients are often afraid. In many ways, their fear becomes routine. It is something I expect, and there is a standard reassuring phrase that I have come to say: 'Don't worry, everything is going to be okay. We're all here to keep you safe.' When I call a patient's fear 'routine', it is not a value judgement, it is just part of ***my*** routine. The sort of work I do doesn't leave a lot of time to work through the origins of fear with a patient. They need something, urgently, and you are required to deliver it, urgently, whether or not they're frightened. So you find yourself focusing on reassurance: '. . . everything is going to be okay, we've got you.' In practical terms, what else can you say?

I'm not sure why, but one day I tried an approach that was less paternalistic in its reassurance. The patient had been thrown through a windscreen in a car accident. He was a young male in his twenties, bleeding, a lot, and I had found myself in the position of keeping my hand pressed on his previously gushing neck-wound.

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'Am I going to die?' he asked suddenly. 'We're doing absolutely everything we can to help you. I know it is hard, but I need you to try and stay calm. Louder: 'But am I going to die?' 'You've got some significant wounds; we're going to do everything we can to make sure that doesn't happen.' Now shouting: 'AM I GOING TO DIE?'

He wanted a definitive: a yes or no, and I could have chosen either, but they would both have been a lie. I had no guarantees. The reality is that some patients look at you through their terrified, dilated pupils and they want you to say it's going to be okay. In the end, either I am right and they will be okay, or I am wrong and I will live with the fact that I chose to tell them otherwise: 'Don't worry, everything is going to be okay. We're all here to keep you safe.' Was I strong enough to cope with the burden of telling patients they would be all right, when actually there was always some chance - however remote - that they might be dead. Maybe it wasn't my job to take away a patient's fear -- usually what was happening to them was frightening . . . But you go to work the next day and stare into the next set of terrified eyes. That patient's fear explodes into the moment that you share together.

4N6 Fanatics

I have distracted frightened children with YouTube videos and teddy bears, so that somebody can put a cannula into their hand. I have stood by an MRI machine with a claustrophobic patient while she had her whole spine scanned and massaged her feet, because it helped her get through the scan. Sometimes, though, patients hand you their fear and you can do absolutely nothing about it.

I met Gloria when I was less than a month qualified. She was ninety years old and had been in hospital for many weeks. She was frail and had become immobile with pneumonia. It was clear that she was in the last days of her life. I remember standing by her bed one morning, when she gripped my hand suddenly and, looking at me with desperation, said, 'Please, don't let me die.'

What could I do? You can't say, 'Of course you are dying, Gloria; you are ninety years old, you have been dying for weeks and I can't change that now.' I tried to explain, to soothe her, but she couldn't hear me through her anguish. As a new doctor, I don't think I expected elderly people to be frightened of dying. I guess it is just easier if they are not; easier for me, anyway.

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I prescribed Gloria medications to help her death pass as comfortably and painlessly as possible, but that didn't seem helpful enough, so I called the consultant. I told him that I thought Gloria needed emotional support. The consultant suggested we should phone and invite the family in that afternoon for a conversation. With apprehension, I asked them if they had any thoughts about how Gloria was feeling. As it turned out, they had already suspected she was frightened, and they thought what would bring Gloria comfort was a chaplain. It seemed like such a straightforward and obvious answer. Why did I not think of it myself? Confronted by her fear, I had panicked because I had no medical solutions.

Gloria died that night, sooner than any of us had expected. I believe the chaplains gave her what she needed - that which I couldn't give her - and I don't think she died frightened. I like to think that Gloria's fear became something else. If you are lucky, I believe that is what happens. Your fear of death becomes an impetus to find comfort. Your fear of losing your hard-fought independence becomes the momentum you need to stand on your feet again. Your fear of becoming the

4NG Fanatics

reason your family must experience grief gives you the strength to seek out truths that may be painful to hear.

And the fear of standing over a patient, with their life in your hands? [That fear] becomes the push you need to know that you did your best. . . . 'Don't worry, everything is going to be okay.

SOURCE INFORMATION

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